

09

LECTURE

HEALTH CARE

MT 1, Second Semester LECTURE 09 - Ms. Rianne Aubrey P. Lopez, RMT, DTA



HEALTH RESOURCES AND SERVICES: INTERNATIONAL AND NATIONAL HEALTH SERVICES

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LEARNING OBJECTIVES

- Understand the Concept of Health Resources and Services
 - Define health resources and services and their significance
 - Identify different types of health services
- Examine International Health Services and Organizations
 - Identify major global health organizations
 - Explain the role of international aid and health programs in global healthcare
 - Discuss how international policies, funding, and partnerships shape national health services
- Analyze the Philippine Healthcare System
 - Describe the structure and function of Philippine Health Services
 - Examine the role of the Department of Health in Public Healthcare
 - Discuss PhilHealth and UHC
- Examine the Role of Local Government Units in Healthcare
 - Explain the responsibilities of LGU's in implementing healthcare services
 - Discuss the role of Barangay Health Centers, Rural Health Units, and Local Hospitals.
 - Identify challenges and improvements in local Healthcare Delivery

I. HEALTH

- Derived from the word **hal**, which means **"hale, sound, whole"**
- As defined by the World Health Organization (WHO) in 1946

- "Health, is a state of complete **physical, mental, and social well-being** and not merely the absence of disease and infirmity"

- This encompasses the highest level of health which involves self-actualization and reaching the completeness of your true potential as one individual would have.

- A **dynamic state** or **condition** of the human organism which means since we are multidimensional, our health is also multidimensional.

- It's not just one thing, it's our physical, emotional, social, intellectual, spiritual, occupational. It's basically our health in nature and it's also a result of our interaction with and how we adapt to our environment.

II. HEALTH SERVICES

- Refers to the organized provision of medical care to individuals or communities.
- It encompasses a very wide range of services aimed at promoting, maintaining, monitoring, or restoring health.

A. QUALITY HEALTH CARE SERVICES

- According to World Health Organization (WHO)
 - Quality Healthcare should be:
 - **Effective** - provides evidence-based healthcare services to those who need them.
 - It means everything we provide as healthcare professionals is **based on logic, facts, and evidence.**
 - **Safe** - avoids harm to the people whose care is intended for.
 - **REMEMBER:** We have the concept of the Hippocratic Oath: "DO NO HARM" As Healthcare Professional, we should ensure that everything that we do or our healthcare services should be safe and should be people-centered.
 - **People-Centered** - provides care that responds to individual preferences, needs, and values.
 - Because we are providing care to the people and to the individual preferences of these people.

B. FACTORS FOR A HEALTH SERVICE TO BE BENEFICIAL

- For a Health Service to be beneficial it must be:
 - **Timely** - reduces wait times and sometimes harmful delays.
 - A lot of the time in healthcare, it is a life-and-death situation, and of course, death would not pause and wait for you to adjust. Because of that, every health service that we provide to our patients should be timely to reduce the wait time and prevent any harmful delay. **ANY TYPE OF DELAY CAN CAUSE MAJOR IRREPARABLE DAMAGES TO OUR PATIENTS.**

- **Equitable** - provides care that does not vary in quality on account of gender, ethnicity, geographic location, and socio-economic status.
- Care would not vary no matter what the socioeconomic status, gender, ethnicity, location, or anything of your patients. It is equal to all.
- **Integrated** - provides care that makes available to the full range of health services throughout the life course.
- Every full range of health services throughout the life course should be given to an individual.
- **Efficient** - maximizes the benefit of available resources and avoids waste.
- When we create waste, that could be the wasted potential that we could have been given to someone else.

C. CATEGORIES OF HEALTH SERVICES

- Promotive Health Services
 - Focused on **enhancing** overall health and well-being by encouraging healthy behaviors and environments.
 - They aim to improve health literacy, empower individuals to make important health decisions. So we are **NOT CURING SOMETHING**, we are **GIVING PEOPLE THE INFORMATION** so that they can live a more healthy life.
 - Example: Health Education Campaigns, Community-Based Nutrition Programs, Smoking Cessation Programs, Road Safety Campaigns, and Mental Health Awareness.
- Preventive Health Services
 - Aimed at preventing the onset of diseases and health conditions through services like immunization, vaccination, health education programs designed to reduce risk factors, and screening

- tests—such as the newborn screening test—to detect potential issues early.
- Vaccination that we may receive is not curative, it is there to help against possible diseases.
- Curative Health Services
 - Involve diagnosis and treatment of existing illnesses or health conditions, with the **goal of curing or managing the disease.**
 - Example: Medical consultations, Surgery, Medication Therapy
- Rehabilitative Health Services
 - Focused on **restoration of a person to normal or near normal function** after a physical or mental illness. (Ex. Physical Therapist and Occupational Therapist)
 - **"NORMAL OR NEAR NORMAL"** means not all rehabilitation program can bring you back to 100%, sometimes the goal of REHAB is to get you at least 70% or 80%
- Palliative Health Services
 - Aim to provide relief from the symptoms and stress of serious illnesses, focusing on improving the quality of life for patients and their families.
 - They address the physical, emotional, and spiritual needs during serious illness or end of life. (Ex. Hospice, Pain Management, Do Not Resuscitate)
 - Aims to improve **QUALITY OF LIFE** suffering from chronic diseases or old age, **not CURE**

III. INTERNATIONAL HEALTH AGENCIES AND SERVICES

- A well established branch of public health with origins in the health situation of developing nations and the efforts of industrialized countries to assist them.
- Basically, these are the first ruled nations assisting the third ruled nation or second ruled nations.

A. Multilateral Donor Health Agencies

- Agencies that represent a group of countries
- **“Multilateral”** means that the funding comes from multiple governments and non-government sources, and is distributed to many countries.

UNITED NATIONS | UN

- Major Multilateral Donor Health Organizations are all a part of the United Nations.
- Is an International Organization founded in 1945.
- The “Peacemakers” of the World
- Currently this is made up of **193 member states** of which the Philippines have been a part of since November 24, 1945.
- There are currently three countries that are part of UN and those three are the **Vatican, Palestine, and Taiwan**

4 MAIN PURPOSES

- To keep peace throughout the world;
- To develop friendly relations among nations;
- To help nations work together to improve the lives of poor people, to conquer hunger, disease, and illiteracy, and to encourage respect for each other's rights and freedoms; and
- To be a center for harmonizing the actions of nations to achieve these goals.

History of the United Nations

April 25–26, 1945

- 50 Countries gathered at the United Nations Conference on International Organization in San Francisco, California.

- For the next 2 months, they proceeded to draft and sign the UN Charter, and this charter then created the UNITED NATIONS, which would help prevent another World War.

October 24, 1945

- The UN officially began after the charter had been ratified by China, France, The Soviet Union, The United Kingdom, The United States and by a majority of other signatory parts of the United Nations.

United Nations Charter

- The founding document of the UN.
- Signed on **June 26, 1945** but it **was not accepted until October 24, 1945**.
- After it was accepted on October 24, 1945 the UN officially began.
- An instrument of international law, and every UN Member State is bound by it.
 - Which means that if you are part of the UN, you must follow the rules written in the Charter.
- It codifies the major principles of international relations, from sovereign equality of States to the prohibition of the use of force in international relations.

Main Bodies of the UN

- **General Assembly**
 - The main deliberative, policymaking and representative organ of the UN.
 - This is the **only UN body with universal representation**, meaning every single one of the 193 member states are represented.
- **Security Council**
 - Has primary responsibility, under the UN Charter, for the **maintenance of international peace and security**.

- It has 15 members; 5 members are permanent and 10 are non permanent.
- **Economic and Social Council**
 - The principle body for **coordination, policy review, policy dialogue** and recommendations on economic, social, and environment issues.
 - Central mechanism for UN Activities.
 - There are 54 elected members.
- **Trusteeship Council**
 - This was established in 1945 under Chapter 13 of the UN Charter.
 - Provide **international supervision for 11 Trust Territories** that had been placed under the administration of 7 Member States.
- **International Court of Justice**
 - The **principal judicial organ** of the United Nations.
 - Its seat is at the Peace Palace in the **Hague, Netherlands**.
 - It's the only one of the 6 principle organs of the United Nations not located in New York.
 - Their role is to settle, in accordance with international law, legal disputes submitted to it by states, and to give advisory opinions on legal questions.
- Secretariat
 - Comprises the Secretary - General and tens of thousands of international UN staff members who carry out the day to day work of the UN as mandated by the General Assembly.
 - The secretary-general is **António Guterres**

17 SUSTAINABLE DEVELOPMENT GOALS

- Adopted by the UN in 2015 to replace the Millenium Development Goals (MDGs).
- Are a Universal Set of Goals, Targets, and Indicators that the UN 193 Member States are using to frame their agendas and political policies through 2030.

- Aims to address inequalities between nations, but also within nations.

17 SUSTAINABLE DEVELOPMENT GOALS	
1.	End poverty in all its forms everywhere
2.	End hunger, achieve food security and improved nutrition, and promote sustainable agriculture
3.	Ensure healthy lives and promote well-being for all at all ages.
4.	Ensure inclusive, equitable and quality education and promote lifelong learning
5.	Achieve gender equality and empower all women and girls.
6.	Ensure access to water and sanitation for all
7.	Ensure access to affordable, reliable, sustainable and modern energy for all
8.	Promote sustained, inclusive and sustainable economic growth, employment, and decent work for all
9.	Build resilient infrastructure, promote inclusive and sustainable industrialization, and foster innovation
10	Reduce inequality within and among countries
11	Make cities inclusive, safe, resilient, and sustainable
12	Ensure sustainable consumption and production patterns
13	Take urgent action to combat climate change and its impacts
14	Conserve and sustainably use the oceans, seas, and marine resources

15	Sustainably manage forest, combat desertification, halt and reverse land degradation, halt biodiversity loss
16	Promote just, peaceful, and inclusive societies
17	Revitalize the global partnership for sustainable development

UNITED NATIONS CHILDREN'S FUND | UNICEF

- Works to **protect the rights of every child**, especially the most disadvantaged and those that are hardest to reach.
 - Protects children from violence and abuse
 - Brings clean water and sanitation to those in need
 - Keeps children safe from climate change and diseases
- Established on **December 11, 1946** in the **aftermath of WWII** (World War II)
 - Provides **long-term** humanitarian and development **assistance to children** that had been devastated by **WWII**
 - Main headquarters in NYC (New York City)
- **World's largest provider of vaccines**

UNITED NATIONS DEVELOPMENT FUND | UNDP

- The lead UN agency on international development
- Established in **1965** by the **General Assembly of the United Nations**

UNDP Mandate

- End poverty
- Build democratic governance
- Rule of law
- Inclusive institutions

4 Focus Areas

- Poverty reduction and millennium development goals

- Democratic governance
- Environment and energy for sustainable development
- Crisis prevention and recovery

FOOD AND AGRICULTURE ORGANIZATION | FAO

- A specialized agency of the UN that leads international efforts to **defeat hunger**
- Established in 1945 in Quebec City, Canada
- Main headquarters in Rome, Italy
- **Primary Concern:** The increased production of food to keep pace with the ever growing world population.

Chief Aims of FAO

- To help nations **raise living standards**
- To **improve the nutrition** of the people of **all countries**
- To **increase** the **efficiency of farming, forestry, and fisheries**
- To better the condition of **rural people**
- To **widen the opportunity** of all people for productive work

Joint United Nations Programme on HIV/AIDS | UNAIDS

- *"Leading the global effort to **end AIDS** as a public health threat by 2030 as part of the Sustainable Development Goals (SDG)"*
- Successor of WHO's Global Program on AIDS
 - Established in 1996
 - Located in Geneva, Switzerland
- Responsible for coordinating efforts to address HIV and AIDS across the UN system

Activities Done by UNAIDS

- Mobilizing leadership and advocacy for effective action on the epidemic
- Providing strategic information and policies to guide global efforts
- Monitoring and evaluation the response to the epidemic

WORLD HEALTH ORGANIZATION | WHO

- The agency that **connects** nations, partners, and people to **promote health**, keep the world **safe** and **serve** the vulnerable—so everyone, everywhere can attain the **highest level of health**
- The most widely recognized governmental health organization
 - Main headquarters in **Geneva, Switzerland**
- Known as the **intergovernmental agency** related to the **UN**
 - Organization formed by two or more nations through a treaty or agreement to address issues of common interest, subject to international law
- Dr. Tedros Adhanom Ghebreyesus**
 - Director General of the WHO

6 Regional Offices of WHO

- Africa (AFRO)**
 - Brazzaville, Congo 🇨🇩
- America (PAHO)**
 - Washington, D.C., USA 🇺🇸
- Eastern Mediterranean (EMRO)**
 - Cairo, Egypt 🇪🇬
- Europe (EURO)**
 - Copenhagen, Denmark 🇩🇰
- Southeast Asia (SEARO)**
 - New Delhi, India 🇮🇳
- Western Pacific (WPRO)**
 - Manila, Philippines 🇵🇭
 - Headed by Dr. Saia Ma'u Piukala

History of WHO

1945

Planning for the WHO began

- The same time UN was planned, they also planned for a governmental health agency
- Occurred when a charter of UN was adopted

1946	Representatives from all the countries in the UN signed the constitution of WHO Happened at the International Conference Law in NYC
April 7, 1948	The WHO Constitution went into force and the organization officially began its work
1952	Jonas Salk develops the inactivated polio vaccine (injection)
1961	Albert Sabin developed the attenuated live-virus vaccine (oral)
1978	WHO sets the aspirational goal "Health for All"
1980	Smallpox is eradicated , following a 12-year global vaccination campaign
1983	HIV is discovered
1999	Global Alliance for Vaccines and Immunizations (now GAVI, the vaccine Alliance) is established First global strategy for the prevention and control of Non-Communicable Diseases (NDCs)
September 2000	The leaders history adopted the UN Millenium Declaration , with a deadline in 2015 Took place at Millenium Summit, a meeting among many world leaders
2000	The WHO Global Outbreak Alert and Response Network (GOARN) was established to detect and combat the international spread of outbreaks
2001	The UN Declaration of Commitment on HIV/AIDS

	Global funds allocated to fight AIDS, Tuberculosis, and Malaria
2008	Heart disease and stroke emerged as the world's no. 1 killers
2009	Emergence of H1N1 Influenza Virus
2014	Ebola outbreak in West Africa
2015	All UN member states adopted the 2030 Agenda for Sustainable Development (SGD)
2018	UN Declaration on Antimicrobial Resistance
2019	UN Declaration on Universal Health Coverage
2020	Global outbreak of Coronavirus Declared a Public Emergency of International Concern
2021	Antiretroviral Therapy was made more accessible for HIV/AIDS - A ground-breaking malaria vaccine was recommended by WHO
2023	75th Anniversary of WHO

6 CORE FUNCTIONS OF WHO

***TAKE TIME TO FAMILARIZE, MIGHT COME OUT!**

- Providing leadership on matters critical to health and engaging in partnerships where joint action is needed
- Shaping the research agenda and stimulating the generation, translation, and dissemination of valuable knowledge

3. Setting norms and standards, and promoting and monitoring what they implemented
4. Articulating ethical evidence-based policy options
5. Providing technical support, catalyzing change, and building sustainable institutional capacity
6. Monitoring health situations and assessing health trends

World Bank

- A vital source of financial and technical assistance to developing countries around the world
 - Made up of 189 members
 - “Working for a world free of poverty”
- AKA the intergovernmental agencies reunited to the to the UN
- Provides low-interest loans, zero to low-interest credits, and grants to developing countries in commercial markets that will lead to the economic growth of a certain country
 - Approved \$495.60 million loan for the Philippine Health System Resilience Project on March 5, 2025
- Aims to rebuild a resilient health system with an emphasis on investments in selected provinces
- They support education, health, public administration, infrastructures, financial and private sector development, agriculture, and environmental resource management.
- Vital source of financial and technical assistance to developing countries around the world.

B. Bilateral Donor Health Agencies

- An official body for a single country or government, which provides aid to developing countries.
 - Provide aid on a country-to-country basis
 - A developed country helps a developing country
- The donor, country, along with their objectives and capacity, provides aid to another country that matches their recipient country's needs
- Only 1 country is providing aid to another country, attempting to match recipient needs with the donor's objectives and capacity to assist, subject to political considerations.

United States Agency for International Development | USAID

- A committed initiative which **works with developing countries** to enhance their systems to **fortify the health and welfare** of international populations
- Focuses specifically on support directed to Sub-Saharan Africa, Asia, Latin America, The Caribbean, Eurasia, and the Middle East
- **Key Prevention Initiatives of USAID**
 - Larger areas of maternal, reproductive, and pediatric health
 - Have specific interests in HIV/AIDS, malaria, and tuberculosis
- **Other countries included in USAID**
 - **Japan International Cooperation Agency (JICA)**
 - **United Kingdom Department for International Development (U.K. DFID)**
 - **Australian Agency for International Development (AusAID)**

*TAKE NOTE: All of these are **first world nations**. You cannot be a nation that can help others if you cannot help yourself. That is why it is always the 1st world*

nations helping or giving services towards a 3rd world nation that needs it. (Ex. Philippines is a developing country which receives a lot of support and aids from these first world nations)

C. INTERNATIONAL NON-GOVERNMENT AGENCIES

- Private agencies that voluntarily use their resources to address a variety of healthcare initiatives
 - **People-to-people aid**
- Views human rights as a fundamental basis for addressing the unseemingly unfelt pain of many people in the world who are suffering
- **Also known as the NGOs**
- Active in development work as the adequacy of the bilateral and multilateral responses become more apparent.

TAKE NOTE: THESE ARE NOT RELATED TO THE GOVERNMENT. THESE ARE PRIVATE INDIVIDUALS WHO ARE FUNDING AND VOLUNTARILY WORKING IN THIS.

International Committee of the Red Cross (ICRC) & Red Crescent Movement

- An impartial, neutral, and independent organization
- **Humanitarian Mission:** Protect the lives and dignity of victims of armed conflict and other situations involving violence, and to provide them with medical assistance.
- The **largest NGO**, comprising 191 National Red Cross and Red Crescent societies.
- Main headquarters in **Geneva, Switzerland**
- **February 1863:** What was to become the International Committee of the Red Cross met for the 1st time in Genova, Switzerland.
- Also known as the **Red Crescent** in countries that do not allow the use of the cross.

7 FUNDAMENTAL PRINCIPLES OF THE ICRC	
Humanity	The need to act in order to prevent and alleviate human suffering
Impartiality	No group of people will be denied with services or receive preferential treatment based on anything other than their needs
Neutrality	Must not take sides or be regarded as doing so, either in speech or actions at any time or place
Independence	Must resist any interference capable of diverting it from embodying the principles of humanity, impartiality, and neutrality (e.g. political, ideological, economic)
Voluntary Service	Individual commitment and devotion to the humanitarian purpose and not inspired by the desire for financial gain
Unity	There can only be one Red Cross or Red Crescent in any country
Universality	All societies have equal status and share equal responsibilities and duties in helping each other

A. MÉDECINS SANS FRONTIÈRES (MSF) / DOCTORS WITHOUT BORDERS

- Founded in **1971** in Paris by a group of journalists and doctors
- A renowned independent medical humanitarian organization

- Provides emergency medical assistance to populations affected by conflicts, epidemics, natural disasters, and exclusion from healthcare in over 70 countries
- Provides health aid to war victims, and assists in other health disasters and development initiatives

Principles of MSF / DOCTORS WITHOUT BORDERS

- Provides assistance to populations in distress, to victims of natural or man-made disasters and armed conflict
- Observes neutrality and impartiality in the name of universal medical ethics and the right to humanitarian assistance and claims full and unhindered freedom in the exercise and its functions
- There is a risk, as these people are sent to legitimate armed conflict war. They understand that there is a risk and do it because they want to. (So scary, but commendable!)

B. OTHER NGOS

1. **Oxford Famine Relief (OXFAM)**
2. **CARE International**
3. **Save the Children International Alliance**

II. OTHER HEALTH AGENCIES

1. **Bill and Melinda Gates Health-Care Planning, Organization, and Evaluation Foundation**
2. **Robert Wood Johnson Foundation**
3. **David and Lucille Packard Foundation**

III. THE PHILIPPINE HEALTH CARE SYSTEM

What is Public Health?

- *"What we as a society do collectively to assure the conditions in which people can be healthy"* (Institute of Medicine, 1988)
- The science and art of preventing disease, prolonging life, promoting physical health and mental health, and efficiency through organized community efforts toward a sanitary environment.

A. History of Public Health in the Philippines

Spanish Colonial Period

- The Philippine health system formally evolved during the Spanish colonial period when the first hospital was built in **Cebu in 1565 (Hospital Real)**
- **1578** - Influential Spanish clergy eventually established the first medical institutions
 - **San Juan de Dios Hospital** - treats disabled, the abandoned, and the poor
 - **San Lazaro Hospital** - care for lepers
- **1800s** - **Smallpox vaccine** was introduced to the country by Spanish Royal Decree

Philippine Revolution and American Colonial Period

- The **Bureau of Public Health** was organized under the revolutionary government established by General Emilio Aguinaldo.
- The Americans took over the reins of the government, eventually constituting the **Board of Health for the Philippine Islands** on July 1, 1901

Philippine Commonwealth

- With the establishment of the Philippine Commonwealth under President Manuel L. Quezon, the **Department of Health and Public Welfare** was organized on May 31, 1939. Though it fell into disarray by the start of WWII
- The incidences of TB, malaria, malnutrition and other diseases increased during the war years
- Once our country was liberated from the Japanese and after the establishment of the republic, the new government had to rebuild everything from the ashes of war.

Establishment of the Department of Health

- **October 4, 1947** - Through **Executive Order No. 94**, the Department of Health was established to what it is today
 - Has supervision over the Bureau of Health, Bureau of Quarantine, Bureau of Hospitals, and all local offices
- **1954** - The **Rural Health Act of 1954** was enacted to transform periculture centers to Rural Health Units
- **1960s** - More medical schools for the nursing and allied professions were established across the country. Many improvements in public health were seen as a result of this.
- With the declaration of martial law in 1972 and with the shift into a parliamentary government in 1978, the DOH was transformed into the **Ministry of Health**.
 - The **Primary Health Care** approach was adopted as a national policy in the late 1970s following the Alma-Ata declaration under Executive Order 852 issued by President Ferdinand Marcos in 1983.
 - It placed the core district hospital and all municipal health offices within the district's catchment area under the supervision of the District Health Officer
- **1986** - Following the People Power Revolution, the Ministry of Health was reorganized as the

DOH under E.O No. 119 signed by President Corazon Aquino on January 30, 1987

IV. COMPONENTS AND SECTORS OF THE PHILIPPINES HEALTH CARE DELIVERY SYSTEM

A. PUBLIC / GOVERNMENT SECTOR

- Consists of the national and local government agencies providing health services
- Health care is generally given free at the point of service
- **2 levels:**
 - **National Level - DOH**
 - **Local Level - LGUs**

Department of Health (DOH)

Kagawaran ng Kalusugan

- Serves as the main governing body of health in the Philippines.
- There are a total of 18 bureaus and services that are responsible for policy development, program planning, standard setting and regulation, and management-related support services.
- **VISION:** "Make Filipinos among the healthiest in Southeast Asia by 2040"
 - To be a global leader for attaining better health outcomes, competitive and responsive health care systems, and equitable health financing.
- **MISSION:** "Lead the country in the development of a productive, resilient, equitable, and people-centered health system"
 - Guarantee equitable, sustainable, and quality health for all Filipinos, especially the poor and to lead the quest for excellence in health

VALUES OF THE DOH	
Integrity	Believes in upholding the truth and pursuing honesty, accountability, and consistency in performing its functions.
Excellence	Continuously strive for the best by fostering innovation, effectiveness and efficiency, pro-action, dynamism, and openness to change.
Compassion & Respect for Human Dignity	Respect for human dignity is encouraged by working with sympathy and benevolence for the people in need.
Commitment	Commits to achieve its vision for the health and development of future generations.
Professionalism	Performs its functions in accordance with the highest ethical standards , principles of accountability, and full responsibility.
Teamwork	Employees work together with a result-oriented mindset.
Stewardship for the Health of People	Pursue sustainable development and care for the environment since it affects the

VALUES OF THE DOH

health of Filipinos.

MAJOR ROLES OF THE DOH

Leader in health

E.O. 102, s. 1999

- **Planning and formulating policies** of health programs and services
- **Monitoring and evaluating** the implementation of health programs, projects, research, training, and services
- **Advocating for health promotion** and healthy lifestyles
- Serving as the **technical authority** in disease control and prevention
- Provides **administrative and technical leadership** in health care financing and implementing the National Health Insurance Law

Enabler and capacity builder

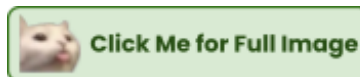
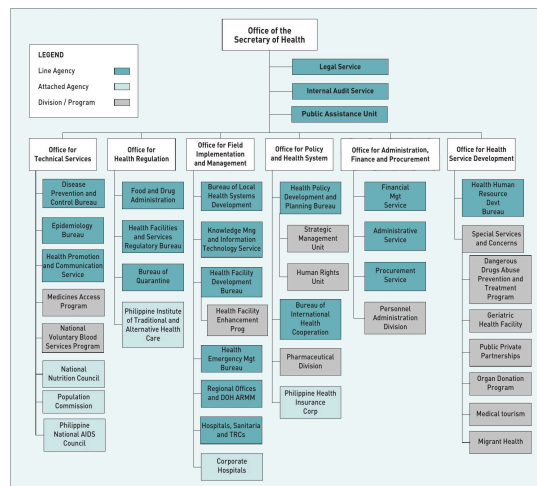
- Providing **logistical support** to LGUs, the private sector, and other agencies in implementing health programs and services
- Serving as the **lead agency** in health and medical research
- Protecting **standards of excellence** in the training and education of health care providers at all levels of the health care system.

Administrator of Specific Services

- **Administrator** of selected health facilities at subnational levels that act as referral centers for local health systems:
 - Tertiary and special hospitals
 - Reference laboratories

- Training centers
- Centers for health promotion
- Centers for Disease Control and Prevention (CDC)
- Regulatory offices
- Provide specific program components for **conditions that affect large segments of the population** (e.g., tuberculosis, malaria, schistosomiasis, HIV/AIDS, and micronutrient deficiencies)
- Develop strategies for responding to emerging health needs
- **Provide leadership** in health emergency preparedness and response services including referral and networking systems for trauma, injury, and catastrophic events

FUNCTIONAL STRUCTURE OF THE DOH



LOCAL HEALTH BOARDS

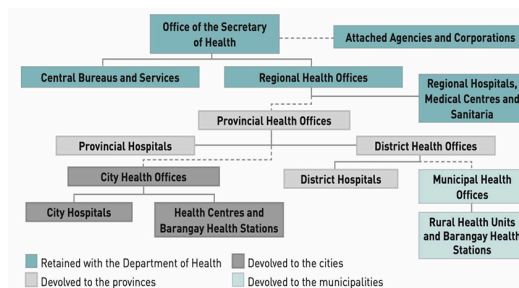
- Consists of 81 provinces, 145 cities (33 are highly urbanized, 5 are independent component cities), 1489 municipalities, and 42,025 barangays

Local Government Code (RA 7160)

- Enacted on **October 10, 1991**
- Mandates the devolution of basic services from the National Government to the LGUs
 - **Devolution** refers to the act in which the national government confers power and authority upon various LGUs to perform specific functions and responsibilities
- Provided for the creation of the Provincial Health Board and the City/Municipality/Local Health Boards

Functions of the Local Health Board

1. Proposing to the Sanggunian **annual budgetary allocations** for the operation and maintenance of health facilities and services within the province/city/municipality
2. Serving as an **advisory committee to the Sanggunian** on health matters
3. Creating committees that **advise local health agencies** on various matters related to health service operations





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RURAL HEALTH UNITS (RHU)

- Commonly known as **Health Center**
- Focuses on preventive and promotive health services and its supervision of **Barangay Health Stations (BHS)** under its jurisdiction
 - The **first contact health care facility** that offers basic services at the barangay level, the satellite station of the RHU
 - Manned by **volunteer Barangay Health Workers (BHW)** under the supervision of the Rural Health Midwife
- DOH Recommendations for Human Resources for Health and Health Facilities Ratio to Population
 - 1 Healthcare Physician: 20,000
 - 1 Public Health Nurse: 10,000
 - 1 Public Health Midwife: 5,000
 - 1 Public Health Dentist: 50,000
 - 1 RHU: 20,000
 - 1 Barangay Health Station: 5,000
 - 1 Barangay Health Worker: 20 Households

V. RURAL HEALTH UNIT PERSONNEL

A. MUNICIPAL HEALTH OFFICER (MHO)

- Also known as **Rural Health Physician**

- Heads the health services at the municipal level

Functions

- **Administrator of the RHU**
 - Prepares municipal health plan and budget
 - Monitors the implementation of basic health services
 - Management of RHU staff
- **Community Physician**
 - Conducts epidemiological studies
 - Formulates health education campaigns on prevention
 - Prepares and implements control measures or rehabilitation measure
- **Medicolegal officer of the municipality**

B. PUBLIC HEALTH NURSE (PHN)

Functions

- Supervises and guides all RHM in the municipality
- Prepares the Field Health Service Information System (FHSIS) quarterly and annual reports of the municipality for submission to the Provincial Health Office
- Utilizes the nursing process in responding to healthcare needs
 - Health education and promotion
- Collaborates with other members of the health team, government agencies, private businesses, NGOs, and people's organizations to address the community's health problems

C. RURAL HEALTH MIDWIFE (RHM) / PUBLIC HEALTH MIDWIFE (PHM)

Functions

- Manages the BHS and supervises and trains BHW
- Provides midwifery services and executes health care programs and activities for women of reproductive age
- Family planning and counseling services
- Conducts patient assessment and diagnosis for a referral for further management
- Performs health information, education, and communication activities
- Organizes the community
- Facilitates the barangay health planning and other community health services

D. RURAL HEALTH INSPECTOR (RHI)

- Wards around to ensure a healthy physical environment in the municipality
- Entails advocacy, monitoring, and regulatory activities
 - Inspection of water supply
 - Inspection of unhygienic household conditions

E. BARANGAY HEALTH WORKER (BHW)

- Considered as the interface between the community and the RHU
- Accredited by the local health board

Functions

- Trained in preventive health care with a strong emphasis on maternal and child care, family planning and reproductive health, nutrition, and sanitation
- Equipped with basic skills for prevention and management of common diseases
- Assists in providing basic services at BHS and RHU

VI. LEVELS OF HEALTH CARE SERVICES AND FACILITIES

- Issued by the DOH through A.O. 2012-0012.
 - Rules and regulations governing the new classification of hospitals and other health facilities in the Philippines.

A. CATEGORY A: PRIMARY CARE FACILITY

- **First contact** health care facility that offers **basic services** including emergency services and provision for normal deliveries
- **With inpatient beds**
 - A **short-stay facility** where the patient spends an average of **1-2 days**
 - e.g., Infirmaries, birthing facilities, etc.
- **Without inpatient beds**
 - **Health centers, outpatient clinics, dental clinics, etc.**

B. CATEGORY B: CUSTODIAL CARE FACILITY

- Provides **long term care** to patients with **chronic conditions** requiring **ongoing health** and **nursing care** due to impairment and reduced degree of independence
 - e.g., custodial psychiatric facilities, drug abuse treatments and rehab centers, sanitarium/leprosaria, nursing homes

C. CATEGORY C: DIAGNOSTIC / THERAPEUTIC FACILITY

- **Examination** of the human body and **specimens** of the human body to **diagnose** and **treat** diseases
 - Also examines water for drinking water analysis
- **Divided into:**
 - **Laboratory facility**
 - Clinical laboratories
 - HIV Testing laboratories
 - Blood service facility
 - Drug testing laboratories
 - Newborn screening laboratory

- Water analysis laboratory (Also examines water for drinking water analysis)
- **Radiologic facility**
- **Nuclear Medicine facility.**
- Regulated by the Philippine Nuclear Resource Institute

D. CATEGORY D: SPECIALIZED OUTPATIENT FACILITY

- Performs **highly specialized procedures** on an **outpatient** basis
- **Medicolegal officer of the municipality**

PRIVATE SECTOR

- Largely market-oriented and health care is **paid through user fees** at the point of service
- The private health sector is regulated by the Government through a system of standards and guidelines implemented through the licensure procedures of the DOH and the accreditation procedures of PhilHealth
- Its involvement in maintaining the people's health includes:
 - Providing Health Insurance
 - Manufacture of medicines, vaccines, medical supplies, equipment, and other products
 - Research and Development
 - Human Resource Development

VII. UNIVERSAL HEALTH CARE IN THE PHILIPPINES

A. UNIVERSAL HEALTH COVERAGE (UHC)

- All individuals and communities receive the health services they need without suffering financial hardships

RA 11223

- "An act instituting Universal Health Care for All Filipinos, prescribing reforms in the health care system, and appropriating funds therefor."
- Signed in **February 2019** by Rodrigo Roa Duterte.
- Covers all Filipinos under the National Health Insurance Program of the Philippines

Objectives

- Provide all Filipinos access to comprehensive and cost-effective health care that covers all spectrums of services
- Covers promotive, preventive, curative rehabilitative, and palliative care

Eligibility and Membership Types

- **All Filipinos are automatically enrolled in PhilHealth** and entitles each person access to:
 - **Individual based care** that can be definitely traced back to one recipient (ambulatory, lab tests, and procedures)

- **Population-based** services

Types of Contributors

- **Direct**
 - Those who are employed
- **Indirect**
 - Those who are unemployed or Persons with Disabilities (PWD)
 - Premiums are shouldered by the government

Source of Funding

- Revenue of government from SinTax Reform Law (R.A. 10351)
- 50% PAGCOR Income
- 40% PCSO Charity Fund
- DOH Funding from the National budget
- PhilHealth subsidy from the national government
- Premium contributors from PhilHealth members (Direct contributors)

Health Promotion and Human Resource

- DOH together with DepED is also required by the UHC Law to foster health consciousness in the basic education programs
 - Current education programs for health professions will be reviewed and new programs will be developed to address the

health needs of the country (2019)

- BSMT, BSN, etc.
- Scholarships will be provided with a Return Service Agreement scheme to supply the growing need
- National rates for salaries of health professionals will be implemented to ensure a competitive labor market in the country and retain an adequate health workforce

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